

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: TMillerPeriod: July 31 to August 30, 2024Report Date: August 31, 2024Coverage: 510/744h (68%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 31 days from Jul 31 to Aug 30

HR

Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	49 bpm	40 bpm	66 bpm
Breathing	18 brpm	11 brpm	44 brpm

Limited data in the final 24h (15/24h); values reflect available readings.

Last 24 hours: one low heart rate episode overnight, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Aug 30	2 AM to 3 AM	1h	1	Elevated Breathing	18	11	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
Aug 30	2 AM to 3 AM	1h	1	Low Heart Rate (brief)	41	40	43	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

42 episodic events Detected, spanning 3 days total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
42	78h	Low Heart Rate	1	68%

Major Findings: Sustained low HR (avg 43, min 40)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Aug 15	2 PM to 3 PM	2h	2	High Breathing	32	16	45	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Aug 22	1 PM to 2 PM	2h	2	High Breathing	35	12	55	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Aug 09	11 AM to 12 PM	24h	10	Low Heart Rate	43	40	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Aug 24	12 PM to 2 PM	28h	12	Low Heart Rate	43	40	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Aug 08	10 AM to 11 AM	6h	6	Elevated Breathing	26	12	53	Check SpO2 and lung sounds; assess for fluid overload or early infection
Aug 20	9 PM to 10 PM	5h	5	Elevated Breathing	25	11	52	Check SpO2 and lung sounds; assess for fluid overload or early infection

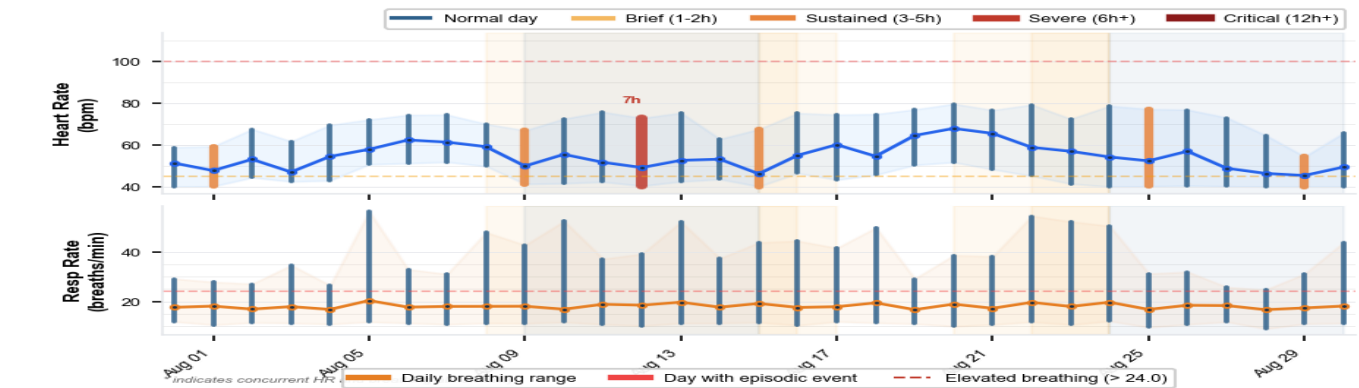
The P5 to P95 heart rate spread was 28 bpm (43 to 71), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 26 brpm, peak 53 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 32 brpm, peak 45 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 28 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

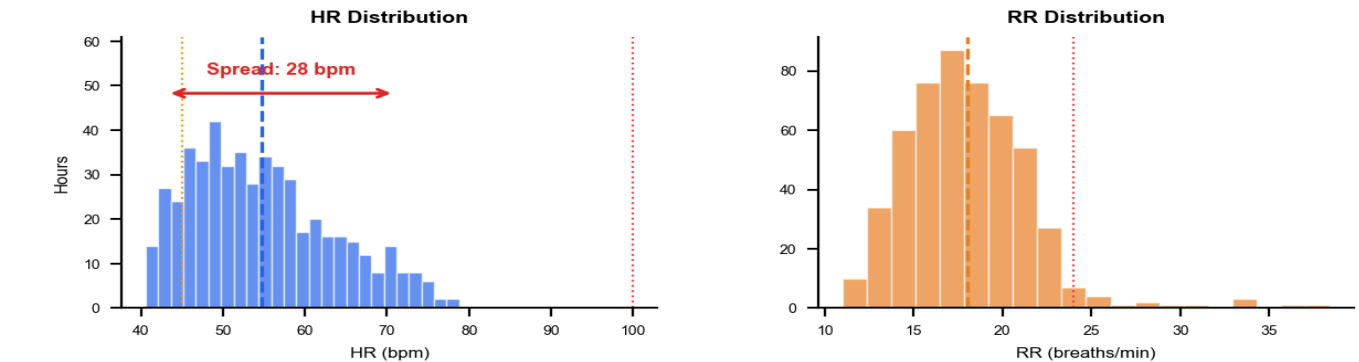
Clinical Pattern Observations:

- 1. **Sustained finding:** 7h continuous Low Heart Rate on Aug 12.
- 2. **High variability:** HR spread 28 bpm (42 to 71).
- 3. **Clustered pattern:** Episodic events on 6 of 31 days (19%).

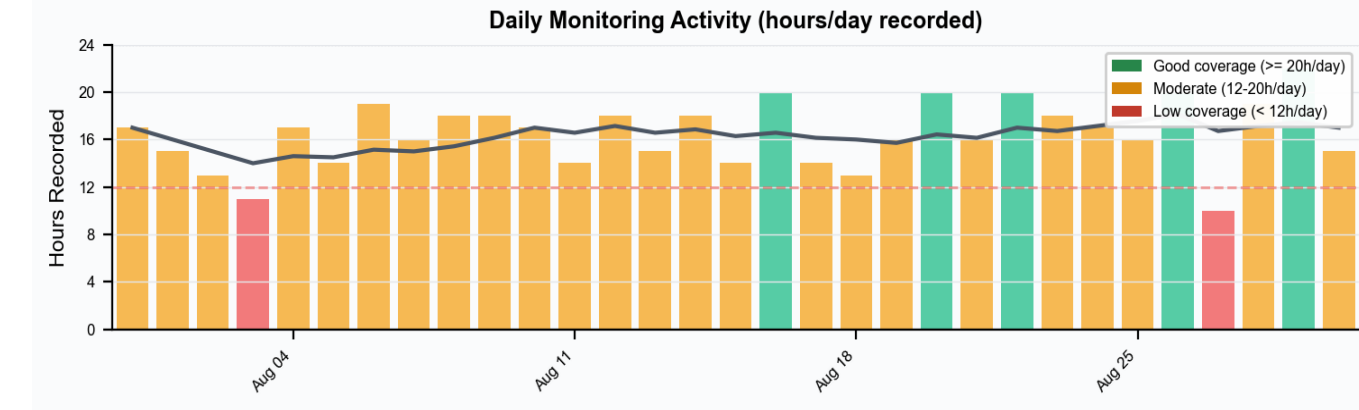


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

HR episode RR episode Recorded, no episode No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.